Centre universitaire de santé McGill

McGill University Health Centre

☐ HME MCH ☒ HGM MGH ☒ HRV RVH
☒ HNM MNH ☒ ITM MCI ☐ CL LC

* F M U – 0 6 3 5 *

Ordonnance d'INSULINE sous cutanée
Dose de base et échelle de correction

Orders for subcutaneous INSULIN
BASELINE DOSE AND CORRECTION SCALE

Consult Endocrinology for patients with Type 1 Diabetes

BASELINE INSULIN DOSES

	Brand of insulin *Physician: Check off only one insulin per box*	**BREAKFAST** or _____ h	**LUNCH** or _____ h	**SUPPER** or _____ h	**BEDTIME** or _____ h
BASAL	LONG-ACTING ☐ **Levemir®** ☐ **Lantus®** INTERMEDIATE ☐ **Humulin N®** (Wardstock) ☐ **Novolin NPH®**	_____ units			_____ units
BOLUS	SHORT ACTING ☐ **Humulin R®**(Wardstock) ☐ **Novolin Toronto®** RAPID ACTING ☐ **Humalog®** ☐ **Novorapid®**	_____ units	_____ units	_____ units	_____ units

INSULIN CORRECTION SCALE: Adjust baseline insulin doses by **adding or subtracting units** as below.

If patient feeding stopped (oral, tube, or TPN): ☐ reduce all baseline doses by ½ ☐ reduce baseline short / rapid acting dose by ½ ☐ use correction scale only ☐ stop all short / rapid acting insulin		**BREAKFAST** or _____ h	**LUNCH** or _____ h	**SUPPER** or _____ h	**BEDTIME** or _____ h **Adjust:**	
Type of insulin to adjust ► For premeal, often same as baseline short or rapid acting		☐ **Same as BOLUS** ☐ **Other:**				
	For glucose levels below 4 mmol/L	****Treat hypoglycemia as per protocol and subtract units as below**				
CAPILLARY BLOOD GLUCOSE (CBG)	**0 – 4 mmol/L****	**- 4 units**	**- 4 units**	**- 4 units**	**- 4 units**	
	4.1 – 6	- 2 units	- 2 units	- 2 units	- 2 units	
	6.1 – 10	**Give Baseline Dose as written above**				
	For glucose levels above 10 mmol/L	**Add units of insulin to baseline dose before meals and at bedtime as noted**			*Check one only* ☐None ☐ Low Dose	☐ High Dose
	10.1 – 12	**+ 2**	**+ 2**	**+ 2**	**+1**	**+ 2**
	12.1 – 14	**+ 4**	**+ 4**	**+ 4**	**+2**	**+ 4**
	14.1 – 16	**+ 6**	**+ 6**	**+ 6**	**+3**	**+ 6**
	16.1 – 18	**+ 8**	**+ 8**	**+ 8**	**+ 4**	**+ 8**
	greater than 18.1 (notify physician)	**+ 10**	**+ 10**	**+ 10**	**+ 5**	**+ 10**

HYPOGLYCEMIC EPISODES ***Cross out unused bedtime correction scale***

When **hypoglycemia** occurs **at above prescribed dosing times or in the interval between** the last dose and the new dosing time:
1. Treat hypoglycemia according to the Hypoglycemia Protocol
2. Decrease the next insulin dose to the 0 - 4 mmol/L range, unless otherwise specified by a physician.

	Nom en lettres moulées *Name in print*	**Signature**	**Service**	**N° Permis** *License* **No**	Heure/Time 00:00	**Date** **AAYY/MM/JD**
Médecin consultant *Consulting physician*						
Médecin traitant *Treating physician*						
Infirmier(ère) *Nurse*						

Original-Dossier médical / Medical Record **Copie jaune-FADM/**Yellow copy-MAR

Insulin types available — onset, peak, duration and Canadian brands

Type	Insulin	Onset	Peak	Duration	Common Canadian brands
Rapid-acting (bolus)	Lispro	10–15 min	1–1.5 h	3–5 h	Humalog (U-100, U-200), Admelog (biosimilar)
	Aspart	10–15 min	1–1.5 h	3–5 h	NovoRapid, Trurapi (biosimilar)
Short-acting (bolus)	Regular	30 min	2–3 h	~6.5 h	Humulin R, Novolin ge Toronto, Entuzity (U-500 KwikPen)
Intermediate-acting (basal)	NPH	1–3 h	5–8 h	Up to 18 h	Humulin N, Novolin ge NPH
Long-acting (basal)	Glargine U-100	~90 min	No pronounced peak	Up to 24 h	Lantus, Basaglar, Semglee
	Detemir	~90 min	No pronounced peak	16–24 h (dose-dependent)	Levemir

Approximate values (Diabetes Canada); they vary with dose, injection site and patient. Toujeo (glargine U-300) is not unit-for-unit equivalent to U-100 glargine. Levemir (detemir) is being discontinued in Canada — confirm availability with pharmacy before starting.

